A participatory approach to determining outcome measures in people with depression

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Depression is a common mental illness that can affect anyone, and whose treatment can take months or years; it is important to get it right. As treatment continues to move from a paternalistic to a more participatory approach, the voices of those living with depression and their informal carers should be a part of the research process.[1]

With insight into how people living with depression view their condition and successful recovery from depression, research and treatment can be more

meaningful and beneficial. The study by Chevance and colleagues[2] intends to provide this insight. Their aim is twofold. They argue that existing scales of depression measurement do not adequately measure what is important to people living with depression or drug decision-makers.[2] Furthermore, they plan to provide a core outcome set that can standardise outcome measurement in future research.[2]

Chevance and colleagues[2] promoted an online survey in three languages (English, French, and German) from April to December 2018, consisting of closed questions to obtain health and demographic information and four open-ended questions intended to obtain qualitative outcome criteria. Including anyone who answered at least one open-ended question, the results comprised information from 1912 people living with unipolar or bipolar depression, 464 informal carers, and 627 healthcare professionals.[2] The authors then coded the 8183 qualitative responses into 80 domains relating to symptom reduction and improved functioning, as well as 57 other domains relating to other aspects of depression, such as stigma reduction and treatment efficacy or safety.[2]

Although people experience many of the same symptoms, depression is a heterogeneous illness, varying in presentation, course, and outcome.[3–5] This heterogeneity is reflected in the diversity of responses: mental symptoms (eg, mental pain (523 [17%] of 3003 participants) and suicidal ideation (695 [23%]), physical symptoms (eg, fatigue (439 [15%]) and disturbed sleep (259 [9%]), and social consequences (eg, social isolation (541 [18%]) and quality of interpersonal relationships (154 [5%]) all feature.[2] It is interesting that health-care professionals and people living with depression cited different concerns as the most frequent: anhedonia for health-care professionals (267 [43%] of 627) and suicide ideation for people living with depression (477 [25%] of 1911).[2] Does this reflect a difference in perspective between those living with the condition and those treating it?

The pragmatic approach in including people with other heath conditions is a strength. People with depression are two to three times more likely to have other long-term conditions than those without depression.[4] To exclude these individuals would not reflect reality. Another strength is the participatory approach. Rather than prompt answers using existing tools, the authors asked people how they felt, using questions derived from a pilot study done with people who live with or treat depression.[2]

The study is a good first step to identify themes for further research, but it could go further. Differences in experiences and priorities might exist between people with different severities or types of depression, which was not explored in this study. In the closed-ended questions, the survey collected much information around antidepressant use, socioeconomic status, and suicide attempts, but none of this is used in any analyses. Additionally, memory bias in those remembering past depressive episodes and unwillingness to share sensitive information, even anonymously, could have affected the answers given.

Research with carefully selected participants is needed to build on the work done and reduce potential biases inherent with self-selecting surveys and purposeful sampling. The authors attempted to increase the study's reach internationally, and had responses from 52 countries.[2] However, the responses were focused on high-income countries, particularly France (1417 [74%] of 1912 patients).[2] It might be that people experience depression somewhat differently.[6] Women were much more likely to answer the survey (1428 [75%]).[2] Although more women are diagnosed with depression than men,[7–8] it is important that men's experiences are also understood. As the study's purpose was to tease out granularity in responses, future research should focus on including more diverse voices.

Seeking and receiving treatment for depression can be arduous, demoralising, and exhausting. Studies focusing on outcomes important to those who benefit—people living with depression, carers, and health care professionals—can help to reflect their needs more accurately. Patient-focused research can in turn ensure people ultimately receive optimal care, further alleviating some of the burden of illness.



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I declare no competing interests.

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